

*** FAX TRANSMISSION ***

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REFERRAL / CONSULTATION REQUEST

Referring Facility: Shenandoah Valley Medical Group
1402 Medical Pkwy, Suite 110
Harrisonburg, VA 22801
Ph: (540) 433-9100 Fax: (540) 433-9188

TO: Johns Hopkins Hospital - Dept. of Neurology
600 N. Wolfe St., Baltimore MD 21287
Attn: New Patient Scheduling / Neurology Consult Line

Patient Name: Whitfield, Gerald R.
DOB: 08/14/1951 Age: 72
MRN (SVMG): SVMG-004481
Insurance: Medicare Part B / AARP Supplement Plan F
PCP: Dr. Constance Berringer, MD (Internal Medicine)

Reason for Referral: Tremor - ? etiology. Referred to Neurology for w/u.

Clinical urgency: [] Emergent [X] Routine [] Urgent
Preferred appt window: Within 6-8 weeks if possible

Subspecialty requested: General Neurology (patient also seen by cards - see attached)

** referring provider uncertain re: exact subspecialty - please route appropriately **

Referring Provider:
Mark T. Okonkwo, DO
Internal Medicine / Primary Care
SVMG - Main Campus
NPI: 1902847563
Direct: (540) 433-9101

Documents included in this fax (12 pages total):

- Referral cover sheet (this page)
- Patient demographics / insurance (p.2)
- Referring provider note 03/14/2024 (p.3-4)
- Prior note - Dr. Berringer 01/09/2024 (p.5)
- Prior note - Neurology walk-in, UVA, 11/02/2023 (p.6-7)
- Prior note - Dr. Okonkwo 08/30/2023 (p.8)

- Labs / imaging (p.9-10)
- Medication list (p.11)
- Discharge summary Rockingham Memorial 02/2024 (p.12)

Authorized signature:

Date: 03/14/2024

Mark T. Okonkwo, DO

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PATIENT DEMOGRAPHICS & INSURANCE

--- EXPORTED FROM SVMG EPIC EHR 03/14/2024 14:02 ---

Full Name: Whitfield, Gerald Raymond

Preferred Name: Jerry

DOB: 08/14/1951

Age: 72 years

Sex: Male

Race/Ethnicity: White / Non-Hispanic

SSN (last 4): ****-**-3317

MRN (SVMG): SVMG-004481

MRN (Rockingham): RMH-29041

Address: 88 Orchard Hill Rd, Dayton VA 22821

Phone (Home): (540) 879-4410

Phone (Cell): (540) 209-3381

Emergency Contact: Whitfield, Sandra (spouse) - same number

Primary Insurance: Medicare Part B

Medicare #: 1EG4-TE5-MK72

Secondary Insurance: AARP Supplemental / UnitedHealthcare Plan F

Group #: 00884 Member ID: 44810339

Auth required?: Yes - referral generated 03/14/2024, Auth# pending

PCP: Constance Berringer, MD - SVMG Internal Medicine

Pharmacy: Kroger Pharmacy, 1781 E. Market St, Harrisonburg VA

*** Patient consented to release of records 01/07/2024 ***

OFFICE VISIT NOTE

Provider: Mark T. Okonkwo, DO - SVMG Internal Medicine
Date of Service: 03/14/2024 Signed: 03/14/2024 16:44
Patient: Whitfield, Gerald R. DOB 08/14/1951 MRN SVMG-004481
Visit type: Established patient - problem-focused

REASON FOR VISIT:

Patient presents today for follow-up of ongoing right hand tremor and to discuss referral to Johns Hopkins per patient's request. Patient was seen here 08/30/2023 for similar complaint. Tremor first noted by patient approximately 18 months ago, possibly longer per wife.

HISTORY OF PRESENT ILLNESS:

Mr. Whitfield is a 72-year-old retired high school principal with a chief complaint of right hand tremor, progressive over the past 1.5 to 2 years. Tremor is described as a shaking of the right hand that is most noticeable when the hand is at rest - e.g., watching television, riding in the car. Patient notes it improves somewhat when he reaches for objects. Wife adds that she has noticed a slight change in his handwriting and that he seems to shuffle a bit when walking, though patient minimizes this. He denies falls. Also endorses mild constipation over the same period, which he has attributed to dietary changes. No significant change in cognition per patient, though wife states he occasionally seems "slower" in conversation. He had a cup of coffee this morning and patient states the shaking "seemed the same" - this was notable to me given I had been considering essential tremor.

He was seen at UVA neurology walk-in in November 2023 (note attached). That encounter documented a resting tremor, right > left, and a recommendation was made to follow up with a movement disorders specialist, however patient was resistant and deferred. No new medications have been started since that visit.

PAST MEDICAL HISTORY:

1. Hypertension - on lisinopril
2. Hyperlipidemia - on atorvastatin
3. Type 2 diabetes mellitus - diet controlled, A1C 6.4% last check
4. GERD - on omeprazole PRN
5. Benign prostatic hypertrophy - on tamsulosin
6. Remote h/o L4-L5 disc herniation (2009, managed conservatively)
7. Appendectomy (1988)
8. Anxiety (longstanding, on lorazepam PRN - prescribed by Dr. Berringer)

FAMILY HISTORY:

Father: deceased, stroke at age 79. Mother: deceased, "shaky hands" in older age - no formal diagnosis documented. Brother: alive, essential tremor diagnosed by neurologist approx 10 years ago. Daughter: alive, no neurologic conditions.

[Page continues - see next page]

NOTE CONTINUED - Whitfield, Gerald R. - 03/14/2024

SOCIAL HISTORY:

Retired principal (retired 2016). Lives with wife in Dayton VA. Non-smoker. Occasional alcohol - states 1-2 beers per week. No illicit drug use. No significant toxic exposures. Drove to appointment today independently.

REVIEW OF SYSTEMS:

Positive: tremor, mild constipation, occasional urinary hesitancy (attributed to BPH), mild fatigue

Negative: no headache, no visual changes, no dysarthria, no dysphagia [NOTE: wife interjected that he has had some "trouble swallowing big pills" - not formally evaluated], no chest pain, no shortness of breath, no weight loss

MEDICATIONS:

See attached medication list (page 11)

ALLERGIES:

Penicillin - rash. Sulfa - unknown reaction.

PHYSICAL EXAMINATION:

General: Pleasant, cooperative male, appearing stated age. Speech is mildly hypophonic.

Vital signs: BP 138/82, HR 68 reg, RR 16, Temp 98.2, SpO2 97% RA, Wt 181 lbs

HEENT: Normocephalic, atraumatic

Neuro: Oriented x3. Cranial nerves grossly intact. Noted resting tremor of right hand, present at rest, 4-5 Hz, pill-rolling character. Left hand - possible trace tremor, examiner uncertain. Rapid alternating movements - mildly slowed bilaterally R>L. Finger-nose-finger - some tremor but no dysmetria. Gait - short-stepped, slightly reduced arm swing on right. Facial expression - mildly reduced.

Musculoskeletal: No rigidity formally tested (outside my scope)

Cardiovascular: RRR, no murmurs

Abdomen: Soft, nontender

ASSESSMENT/PLAN:

1. Tremor, right > left, resting - differential includes essential tremor vs. early parkinsonian syndrome vs. drug-induced (though med review does not strongly suggest this - metoclopramide not on list). Family history of essential tremor in brother makes ET possible; however, rest predominance and associated exam findings give me pause. Deferring to neurology.

- Referral placed to Johns Hopkins Neurology (patient preference)

- Advised patient not to start any new medications for tremor prior to neurology eval

2. HTN: continue lisinopril 10mg daily, BP adequate today

3. Hyperlipidemia: continue atorvastatin 40mg

4. BPH: tamsulosin continued
5. Anxiety: lorazepam PRN - discussed with patient - will not adjust at this time pending neuro eval
6. DM2: diet controlled, no changes
7. F/u in 3 months or sooner if needed

Electronically signed: Mark T. Okonkwo, DO 03/14/2024 16:44

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PRIOR OFFICE NOTE

Provider: Constance Berringer, MD - SVMG Internal Medicine
Date of Service: 01/09/2024 Signed: 01/09/2024 20:11
Patient: Whitfield, Gerald R. DOB 08/14/1951 MRN SVMG-004481

** NOTE: Portions of history copied forward from 08/30/2023 encounter **

CHIEF COMPLAINT:

Annual wellness visit + patient follow-up re: tremor and recent ER visit (see discharge summary).

HPI:

Mr. Whitfield presents for his annual Medicare wellness visit. He and his wife also wish to discuss his tremor, which per wife has "definitely gotten worse" since last seen here in August. He was admitted briefly to Rockingham Memorial in late January 2024 [NOTE: this is inconsistent - visit is 01/09, discharge summary is 02/2024, possible the wellness visit predates admission or patient/wife misstated] after a fall in the bathroom. He reports he did not lose consciousness. He denies vertigo. Tremor continues. He was seen at UVA neurology walk-in in November 2023 - report states resting tremor, recommendation for movement disorders follow-up.

PAST MEDICAL HISTORY:

Hypertension, hyperlipidemia, DM type 2 (diet controlled), GERD, BPH, anxiety, prior disc herniation L4-L5

[same as prior notes - see 08/2023 note for full history]

PHYSICAL EXAM (pertinent):

Neuro: Tremor present right hand, mild, resting > kinetic. Gait somewhat cautious. Mild facial masking noted by this examiner - not noted in prior note 08/2023. Balance on tandem gait testing - fair, mild difficulty.

BP: 132/78 HR: 71 Weight: 178 lbs (down 3 lbs from Aug)

A/P:

1. Tremor - ? ET vs other. Brother with ET. Family history positive. Continue to monitor. I have noted some features that are atypical for ET - primarily the resting character and gait changes. Will defer to neurology. Referral to Hopkins requested by patient.
2. Fall - single episode, in bathroom, likely mechanical. No syncope. Orthostatics checked - negative. Will monitor.
3. Anxiety - lorazepam refilled, patient using 0.5mg approx 3x/week
4. GERD - omeprazole 20mg PRN, patient uses sparingly
5. Annual labs ordered (see attached)
6. Return in 6 months

Signed: C. Berringer MD 01/09/2024 20:11

PRIOR CONSULTATION NOTE - OUTSIDE FACILITY

Facility: University of Virginia Health - Neurology Walk-In Clinic
Provider: Adaeze Nwosu, MD - Neurology Resident, PGY-3
Attending: Philip Hennessey, MD - General Neurology
Date of Service: 11/02/2023 Signed: 11/02/2023 21:45
Patient: Whitfield, Gerald R. DOB 08/14/1951 MRN UVA: U-88204417

** FAX OF FAX - image quality degraded **

CHIEF COMPLAINT:

72M self-referred for right hand tremor x approximately 1 year.

HPI:

Mr. Whitfield is a 72-year-old male, retired educator, who presents to the UVA neurology walk-in clinic for evaluation of a right-hand tremor that he first noticed approximately 12-14 months ago. He describes it as most prominent when his hand is "just sitting there." He initially attributed this to caffeine intake and attempted to reduce coffee consumption without significant improvement. He notes his wife has commented on a change in his walking and handwriting. He denies any head tremor. He denies alcohol use as a contributor. He has not trialed propranolol or primidone. He has a brother with a documented diagnosis of essential tremor.

He takes lorazepam 0.5mg PRN for anxiety (prescribed by PCP) - frequency of use not fully characterized at this visit [NOTE: lorazepam may be masking some anxiety-related component; also could theoretically blunt tremor amplitude per Dr. Hennessey's verbal comment].

NEUROLOGICAL EXAM:

Mental status: Alert and oriented x4. Speech somewhat low volume. MMSE not formally administered.

Cranial nerves: II-XII grossly intact. Facial expression mildly reduced - mild hypomimia.

Motor: 5/5 throughout. Mild cogwheel rigidity right wrist on passive ROM - present with distraction maneuver. [attending confirmed]

Tremor: 4-5 Hz resting tremor right hand with pill-rolling quality. Re-emergent with arm extension after latency of ~2 sec. No head or voice tremor. Left side - trace only.

Cerebellar: Finger-nose-finger intact. No dysdiadochokinesia. Heel-shin intact.

Gait: Mildly decreased arm swing right side. Shortened stride. No festination observed. Turns en bloc.

Reflexes: 2+ symmetric throughout. Plantar responses flexor bilat.

[Note continues p.7]

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NOTE CONTINUED - UVA Neurology 11/02/2023 - Whitfield, Gerald R.

IMPRESSION:

72-year-old male with right-hand resting tremor with pill-rolling character, re-emergent tremor on arm extension, mild cogwheel rigidity right wrist, reduced arm swing and shortened stride on gait, and mild hypomimia. Family history of essential tremor in sibling.

The clinical picture is not straightforwardly consistent with essential tremor in this examiner's opinion, given the rest predominance, cogwheeling, and gait changes. However, the attending and I discussed that early ET can occasionally present with some resting component, and the family history is notable. Scans were not obtained at this visit given the walk-in setting and lack of insurance pre-authorization.

DaTscan was discussed as a potential diagnostic tool but not ordered at this time.

PLAN:

1. Recommend formal follow-up with a movement disorders subspecialist for further characterization.
2. Hold off on empiric treatment with propranolol or carbidopa-levodopa until subspecialty evaluation.
3. MRI brain without contrast suggested - no acute contraindication. Could be arranged by PCP.
4. Patient provided with UVA movement disorders referral paperwork but stated he may seek care closer to home or at Hopkins.

Attending co-signature: Philip Hennessey, MD 11/02/2023

Electronically signed: Adaeze Nwosu, MD 11/02/2023 21:45

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PRIOR OFFICE NOTE

Provider: Mark T. Okonkwo, DO - SVMG Internal Medicine
Date of Service: 08/30/2023 Signed: 08/30/2023 17:22
Patient: Whitfield, Gerald R. DOB 08/14/1951 MRN SVMG-004481

** COPY-FORWARDED NOTE - original in SVMG chart 08/30/2023 **

CHIEF COMPLAINT:

New complaint - right hand tremor x 6-8 months

HPI:

Mr. Whitfield is a 71-year-old male who presents today with a right hand tremor that he first noticed approximately 6-8 months ago. He initially attributed it to fatigue or stress. He is retired. Wife accompanies. Patient reports it is worse when sitting quietly. He denied any change in gait or writing at THIS visit [contrast with later notes where writing change is acknowledged]. He drinks 2 cups of coffee per day; does not believe this is the cause. He denies headache, vision changes, or other neurological symptoms. He has a sibling with "shaky hands" - formal diagnosis unknown at this time.

EXAM (pertinent):

Neuro: Alert, appropriate. CN II-XIII intact. Right hand tremor visible at rest, low amplitude, approximately 4 Hz. Gait: normal [note: later visits document gait abnormality - possible progression or prior examiner not testing gait formally]. Rapid alternating movements: intact.

BP: 144/88 HR: 74 Wt: 183 lbs

ASSESSMENT:

Right hand tremor - differential: essential tremor (family history, caffeine), physiologic tremor, less likely medication induced (current meds reviewed - none classically tremorigenic though lorazepam chronic use noted). Anxiety may be contributing.

PLAN:

1. Labs - TSH, CMP, CBC (results in chart - see attached)
2. Discussed trial of propranolol - patient declined at this time, wishes to monitor
3. Neurology referral offered - patient declined, wishes to wait and see
4. Reduce caffeine if possible
5. Return in 3-4 months

Signed: M.T. Okonkwo DO 08/30/2023 17:22

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LABORATORY & IMAGING RESULTS

Patient: Whitfield, Gerald R. DOB 08/14/1951 MRN SVMG-004481

CBC (Ordered 08/30/2023 - resulted 08/31/2023)

WBC	7.2 K/uL	[4.5-11.0]	Normal
RBC	4.55 M/uL	[4.2-5.8]	Normal
Hgb	13.8 g/dL	[13.0-17.0]	Normal
Hct	41.2 %	[38-50]	Normal
MCV	88.4 fL	[80-100]	Normal
PLT	211 K/uL	[150-400]	Normal

CMP (Ordered 08/30/2023 - resulted 08/31/2023)

Na	139 mEq/L	[136-145]	Normal
K	4.1 mEq/L	[3.5-5.1]	Normal
Cl	102 mEq/L	[98-107]	Normal
CO2	25 mEq/L	[22-29]	Normal
BUN	18 mg/dL	[8-23]	Normal
Creat	1.0 mg/dL	[0.7-1.3]	Normal
Gluc	108 mg/dL	[70-100]	HIGH *
Ca	9.3 mg/dL	[8.4-10.2]	Normal
AST	27 U/L	[10-40]	Normal
ALT	31 U/L	[7-56]	Normal
AlkPhos	88 U/L	[44-147]	Normal

TSH (Ordered 08/30/2023 - resulted 08/31/2023)

TSH	2.14 uIU/mL	[0.40-4.50]	Normal
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HbA1c (Ordered 01/09/2024 - resulted 01/11/2024)

HbA1c	6.4 %	[<5.7 normal, 5.7-6.4 prediabetes]	BORDERLINE
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LIPID PANEL (01/09/2024)

Total Chol	172 mg/dL	[<200]	Normal
LDL	94 mg/dL	[<100]	Normal
HDL	48 mg/dL	[>40]	Normal
Triglyc	149 mg/dL	[<150]	Normal

Vitamin B12 (01/09/2024)

B12	388 pg/mL	[200-900]	Normal
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RPR (01/09/2024) - NONREACTIVE

Urinalysis (01/09/2024) - unremarkable

[Imaging on next page]

IMAGING - Whitfield, Gerald R. - continued

MRI BRAIN WITHOUT CONTRAST

Facility: Rockingham Memorial Hospital - Radiology

Date: 02/08/2024 Read by: Anita Patel, MD - Radiology

Ordering provider: Okonkwo, M.T.

TECHNIQUE: MRI of the brain was performed without IV contrast using standard protocols including axial FLAIR, DWI, T1, T2, and susceptibility-weighted imaging (SWI).

FINDINGS:

Brain parenchyma: Mild, age-appropriate cortical atrophy. Mild periventricular and subcortical white matter T2/FLAIR signal changes consistent with chronic small vessel ischemic disease, Fazekas grade 1-2. No focal lesion, acute infarct, or hemorrhage identified. No mass effect or midline shift.

Basal ganglia/substantia nigra: No gross structural abnormality identified on conventional MRI. [NOTE: SWI imaging reviewed - no frank abnormality, though nigrosome-1 assessment is limited on 1.5T without dedicated sequences].

Ventricles: Mildly enlarged, proportionate to degree of atrophy.

Posterior fossa: Cerebellum and brainstem unremarkable.

Vascular flow voids: Preserved.

IMPRESSION:

1. Mild age-related cortical atrophy.
2. Mild periventricular and deep white matter signal changes consistent with chronic small vessel ischemic disease; no evidence of acute infarct.
3. No intracranial mass, hemorrhage, or hydrocephalus.
4. MRI is not sufficiently sensitive to rule out early nigrostriatal dysfunction; clinical and functional imaging correlation advised if parkinsonian syndrome is suspected.

Electronically signed: A. Patel MD 02/08/2024 13:19

CARDIAC WORKUP (included per cardiology attachment - may not be directly relevant)

ECG 01/10/2024: Normal sinus rhythm, rate 69. No acute ST-T changes. QTc 432ms.

Echocardiogram 01/15/2024 (transthoracic):

EF: 60-65%. Normal LV size and function. Mild aortic sclerosis without stenosis.

Diastolic dysfunction grade I. No significant valvular abnormality. No wall motion abnormality.

[Ordered by cardiologist - Dr. Sanjay Mehta - f/u re: HTN management. Per cardiology note, patient was seen for hypertension; tremor mentioned incidentally.]

DaTscan: Not performed.

CURRENT MEDICATION LIST

Patient: Whitfield, Gerald R. DOB 08/14/1951 MRN SVMG-004481

Reconciled by: SVMG nursing staff 03/14/2024

** NOTE: Patient self-reports some OTC meds not always listed - see notes **

PRESCRIPTION MEDICATIONS:

1. Lisinopril 10mg PO daily (Hypertension)
2. Atorvastatin 40mg PO daily at bedtime (Hyperlipidemia)
3. Tamsulosin 0.4mg PO daily (BPH)
4. Omeprazole 20mg PO PRN (max 1x/day) (GERD)
5. Lorazepam 0.5mg PO PRN anxiety (Anxiety - prescribed by Dr. Berringer)
[frequency of use per pt report: ~3x/week; wife suggests may be more frequent]
6. Aspirin 81mg PO daily (Cardiovascular prophylaxis - patient self-initiated, endorsed by PCP)

OVER-THE-COUNTER / SUPPLEMENTS (patient-reported):

7. Melatonin 5mg PO at bedtime PRN sleep
8. Fish oil 1000mg PO daily
9. Magnesium oxide 400mg PO daily (Patient started for leg cramps, approx 6mo ago)
10. Vitamin D3 2000 IU PO daily
11. Ibuprofen 400mg PRN musculoskeletal pain (Infrequent use per patient; last use ~2 weeks ago for knee pain)
12. MiraLAX (polyethylene glycol) PRN constipation (Recent addition - constipation noted in HPI)

DISCONTINUED / HISTORICAL:

Metformin 500mg - discontinued 2021 (patient preference; glucose diet controlled)

Hydrochlorothiazide 12.5mg - discontinued 2022 (hyponatremia, switched to lisinopril)

ALLERGIES:

Penicillin - rash
Sulfa - unknown/uncharacterized

No antipsychotics, metoclopramide, or dopamine-blocking agents currently on list.

No antiparkinsonian medications on list.

DISCHARGE SUMMARY

Facility: Rockingham Memorial Hospital
Admit Date: 02/05/2024 Discharge Date: 02/07/2024
Patient: Whitfield, Gerald R. DOB 08/14/1951 MRN RMH-29041
Admitting Dx: Fall, bathroom, with minor head laceration
Discharge Dx: Mechanical fall; closed head injury without LOC; laceration scalp, repaired
Attending: Priya Sundaram, MD - Hospitalist

REASON FOR ADMISSION:

72-year-old male admitted after unwitnessed fall in bathroom at home in early morning hours. Wife heard him fall; found him on floor. No reported LOC. He struck his head on the sink edge, resulting in a 2 cm scalp laceration, repaired with staples in the ED. GCS 15 on arrival.

HOSPITAL COURSE:

Patient admitted for observation given head trauma and age. CT head without contrast performed in ED - no intracranial hemorrhage, no fracture. Orthostatics negative. Neurological exam during admission notable for the following (neurology not formally consulted during this admission - note by hospitalist only):

Resting tremor right hand - present, patient endorses longstanding, per chart ongoing >1yr

Gait: cautious, slow, slightly shuffling per nursing documentation on 02/06 AM ambulation assessment

Patient did not report prodrome before fall. Possible contribution from nighttime lorazepam use (took 0.5mg at ~11PM per patient report).

Cardiology consulted for hypertension management during admission (Dr. Mehta). Echo and ECG obtained (see labs/imaging section). No cardiac etiology for fall identified.

MRI brain ordered prior to discharge by Dr. Sundaram - performed 02/08 as outpatient (results attached, see p.10).

DISCHARGE CONDITION: Good

DISCHARGE MEDICATIONS:

Continued home medications (see med list). No new Rx added.

Staples: removal in 7-10 days by PCP.

FOLLOW-UP:

1. PCP (Dr. Okonkwo or Dr. Berringer) within 1-2 weeks
2. Neurology referral strongly recommended by hospitalist and cardiology - tremor and gait noted as concerning during admission. Per Dr. Mehta verbal: "someone needs to get a neurologist to look at this man's gait."

3. Discuss fall prevention and home safety assessment with PCP
4. Patient and wife counseled re: fall precautions, bathroom grab bars, nighttime lighting

DISCHARGE DIET:

Regular / diabetic-friendly

Discharge Summary signed: Priya Sundaram, MD 02/07/2024 14:33

-- END OF REFERRAL PACKET -- Page 12 of 12 --